

|                            |                         |                           |                    |
|----------------------------|-------------------------|---------------------------|--------------------|
| <b>Patient Name</b>        | : MS.CHIANA MIKOVA      | <b>Registered On</b>      | : 30-04-2024 19:50 |
| <b>Patient Id</b>          | : 15206                 | <b>Sample Received On</b> | : 30-04-2024:17:00 |
| <b>Age/DOB/Gender</b>      | : 47Y/1977-01-24/Female | <b>Reported On</b>        | : 30-04-2024 20:44 |
| <b>Nationality</b>         | : -                     | <b>Sample UID No.</b>     | : D002B042001472   |
| <b>Customer Type</b>       | : Collection Center     | <b>Customer Name</b>      | : AIMS             |
| <b>Ref. Doctor Name</b>    | : -                     | <b>Patient UID No.</b>    | : -- (Other)       |
| <b>External Patient Id</b> | : 71367                 |                           |                    |
| <b>Print Version</b>       | : v.1                   |                           |                    |

### LIVER FUNCTION TEST

| <u>Investigation</u>                                                                             | <u>Result</u> | <u>Units</u> | <u>Biological Reference Interval</u>                                                                                                                             |
|--------------------------------------------------------------------------------------------------|---------------|--------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>ALT (SGPT)</b><br>Sample Type :Serum<br>Method : ALTv- VITROS                                 | 16            | U/L          | 7-45                                                                                                                                                             |
| <b>AST (SGOT)</b><br><br>Sample Type :Serum<br>Method : AST- Vitros                              | 29            | U/L          | Males<br>0-11 months: not established<br>1-13 years: 8-60<br>>=14 years: 8-48<br>Females<br>0-11 months: not established<br>1-13 years: 8-50<br>>=14 years: 8-43 |
| <b>ALKALINE PHOSPHATASE</b><br>Sample Type :Serum                                                | 58            | U/L          | 35-104                                                                                                                                                           |
| <b>GAMMA GT SERUM</b><br>Sample Type :Serum<br>Method : Vitros Microslide                        | 21            | U/L          | 4-44                                                                                                                                                             |
| <b>BILIRUBIN TOTAL SERUM</b><br>Sample Type :Serum<br>Method : Diphylline, Diazonium Salt-VITROS | 0.3           | mg/dL        | 0.1-1.3                                                                                                                                                          |
| <b>BILIRUBIN DIRECT</b><br>Sample Type :Serum<br>Method : Spectrophotometer                      | 0.1           | mg/dL        | 0-0.3                                                                                                                                                            |
| <b>BILIRUBIN INDIRECT</b><br><br>Sample Type :Serum<br>Method : Direct measured                  | 0.2           | mg/dL        | 0-6 days: 0.1 – 1.0<br>7-14 days: < 15.0<br>15 days to 17 years: < 1.0<br>>=18 years: > 1.2 mg/ dL                                                               |
| <b>TOTAL PROTEIN SERUM</b><br>Sample Type :Serum<br>Method : Calculated                          | 6.9           | g/dL         | 6.3-8.2                                                                                                                                                          |
| <b>ALBUMIN SERUM</b><br>Sample Type :Serum                                                       | 4.2           | g/dL         | 3.5-5.0                                                                                                                                                          |



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| <b>Ref. Doctor Name</b>    | : -                     | <b>Patient UID No.</b>    | : -- (Other)       |
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Method : Dye Binding BCG

|                     |      |      |         |
|---------------------|------|------|---------|
| <b>GLOBULIN</b>     | 2.70 | g/dL | 2.3-3.5 |
| Sample Type :Serum  |      |      |         |
| Method : Calculated |      |      |         |

|                               |      |    |         |
|-------------------------------|------|----|---------|
| <b>ALBUMIN GLOBULIN RATIO</b> | 1.56 | -- | 1.2-1.8 |
| Sample Type :Serum            |      |    |         |
| Method : Calculated           |      |    |         |

#### Interpretation -

Hepatic function panel results are not diagnostic of a specific condition; they indicate that there may be a problem with the liver. In a person who does not have symptoms or identifiable risk factors, abnormal liver test results may indicate a temporary liver injury or reflect something that is happening elsewhere in the body-such as in the skeletal muscles, pancreas, or heart. It may also indicate early liver disease and the need for further testing and periodic monitoring.

Results of liver panels are usually evaluated together. Several sets of results from tests performed over a few days or weeks are often assessed together to determine if a pattern is present. Each person will have a unique set of test results that will typically change over time. A healthcare practitioner evaluates the combination of liver test results to gain clues about the underlying condition. Often, further testing is necessary to determine what is causing the liver damage or disease.



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**Patient Id** : 15206  
**Age/DOB/Gender** : 47Y/1977-01-24/Female  
**Nationality** : -  
**Customer Type** : Collection Center  
**Ref. Doctor Name** : -  
**External Patient Id** : 71367  
**Print Version** : v.1

**Registered On** : 30-04-2024 19:50  
**Sample Received On** : 30-04-2024:17:00  
**Reported On** : 30-04-2024 20:44  
**Sample UID No.** : D002B042001475  
**Customer Name** : AIMS  
**Patient UID No.** : -- (Other)

### COMPLETE BLOOD COUNT (CBC)

| Investigation             | Result      | Units                 | Biological Reference Interval |
|---------------------------|-------------|-----------------------|-------------------------------|
| HAEMOGLOBIN               | 13.1        | g/dL                  | 12-16                         |
| HEMATOCRIT                | 37.5        | %                     | 33-51                         |
| RBC COUNT                 | <b>3.87</b> | X 10 <sup>6</sup> /μL | 4.00-5.20                     |
| MCV                       | 96.9        | fL                    | 77-100                        |
| MCH                       | 34          | Pg                    | 26-34                         |
| MCHC                      | 35.1        | g/dL                  | 32-36                         |
| RDW-CV                    | 15.4        | %                     | 11.5-17                       |
| PLATELET COUNT            | 213         | x10 <sup>3</sup> /ul  | 150-450                       |
| MPV                       | <b>12.7</b> | fL                    | 7.5-12.0                      |
| TOTAL LEUKOCYTE COUNT     | 8.7         | x10 <sup>3</sup> /ul  | 4.5-11.0                      |
| NEUTROPHIL                | 44.3        | %                     | 40-73                         |
| LYMPHOCYTE                | 40.8        | %                     | 25-45                         |
| MONOCYTE                  | 9.9         | %                     | 4-12                          |
| EOSINOPHIL                | 4.2         | %                     | 0-7                           |
| BASOPHIL                  | 0.8         | %                     | 0-2                           |
| ABSOLUTE NEUTROPHIL COUNT | 3.85        | x10 <sup>3</sup> /ul  | 1.5-7.0                       |
| ABSOLUTE LYMPHOCYTE COUNT | 3.53        | x10 <sup>3</sup> /ul  | 1.1-5.0                       |
| ABSOLUTE EOSINOPHIL COUNT | 0.36        | x10 <sup>3</sup> /ul  | 0.15-0.5                      |
| ABSOLUTE MONOCYTE COUNT   | <b>0.85</b> | x10 <sup>3</sup> /ul  | 0.2-0.8                       |
| ABSOLUTE BASOPHIL COUNT   | 0.07        | x10 <sup>3</sup> /ul  | 0-0.15                        |

Sample Type : EDTA Whole Blood

### Interpretation -

**Method:** EDTA Whole Blood: Tests done on Automated Five Part Cell Counter. (Hb by Photometry method .RBC & PLT by Electric Impedance, PCV by Numeric Integration method. WBC and Differential count by Double Hydrodynamic Sequential System (DHSS). Other parameters Calculated.) All Abnormal Haemograms are reviewed confirmed microscopically.

Disclaimer:



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- 1) The above result relate only to the specimens. Received and tested in laboratory and should be always correlate with clinical findings and other laboratory markers.
- 2) Improper specimen collection, handling. Storage and transportation may result in false negative/Positive results.

#### Comments:

A complete blood count (CBC) test is a commonly performed blood test that provides important information about the components of your blood. It measures various parameters related to red blood cells, white blood cells, and platelets.

**Useful for :** Detecting and diagnosing medical conditions, Preoperative assessment, Detecting and diagnosis disorders of RBCs, WBCs & Platelets. As a Screening tool to confirm a hematologic disorder, to establish or rule out a diagnosis, to detect an unsuspected hematologic disorder, or to monitor effects of radiation or chemotherapy.

**Reference:** Horiba Yumizen 550, Performance and Reference: Tools for Accreditation 3.4.15. Reference Values, page 47



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#### AMYLASE SERUM

| <u>Investigation</u>                      | <u>Result</u> | <u>Units</u> | <u>Biological Reference Interval</u> |
|-------------------------------------------|---------------|--------------|--------------------------------------|
| <b>AMYLASE SERUM</b>                      | 45            | U/L          | 37-125                               |
| Sample Type :Serum                        |               |              |                                      |
| Method : Amylopectin, colorimetric-VITROS |               |              |                                      |

#### Interpretation -

**Increased levels** : Parotitis, pancreatitis, intestinal obstruction or infarction, strangulated bowel, ectopic pregnancy, perforated hollow viscus, biliary tract disease of all types, diabetic ketoacidosis, pancreatic cyst and pseudocyst, peritonitis, macroamylasemia, some lung and ovarian tumors, renal failure, ERCP, abdominal trauma, head injury, viral infections, postoperative patients, alcohol

**Decreased levels** : Pancreatic insufficiency, advanced cystic fibrosis, severe liver disease, pancreatectomy



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| <b>Ref. Doctor Name</b>    | : -                     | <b>Patient UID No.</b>    | : -- (Other)       |
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### Apo A1

| <u>Investigation</u>                | <u>Result</u> | <u>Units</u> | <u>Biological Reference Interval</u> |
|-------------------------------------|---------------|--------------|--------------------------------------|
| <b>Apo A1</b>                       | 155.07        | mg/dL        | 105-175                              |
| Sample Type :Serum                  |               |              |                                      |
| Method : Immunoturbidimetric method |               |              |                                      |

### **Comments:**

Apolipoprotein A1 is one of the apoproteins of high density lipoproteins (HDL) which is inversely related to the risk of CAD. Individuals with Tangier disease have < 1% of normal Apo A1. Levels <90mg/dL indicate increased risk of Atherosclerotic disease.



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| <b>Customer Type</b>       | : Collection Center     | <b>Customer Name</b>      | : AIMS             |
| <b>Ref. Doctor Name</b>    | : -                     | <b>Patient UID No.</b>    | : -- (Other)       |
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### Apo B

| <u>Investigation</u>                | <u>Result</u> | <u>Units</u> | <u>Biological Reference Interval</u> |
|-------------------------------------|---------------|--------------|--------------------------------------|
| <b>Apo B</b>                        | 89.21         | mg/dL        | 60-140                               |
| Sample Type :Serum                  |               |              |                                      |
| Method : Immunoturbidimetric method |               |              |                                      |

### **Interpretation -**

Apolipoprotein B is a more powerful independent predictor of Coronary Heart Disease (CAD) than LDL Cholesterol. It is useful in assessing the risk of CAD and to classify Hyperlipidemias. Apolipoprotein studies help in monitoring coronary bypass surgery patients with regard to risk and severity of re-stenosis. They are also useful in assessing risk of re-infarction in patients of Myocardial infarction. As per recommendations of National Cholesterol Education Program (NCEP) the clinical significance of results is as follows:  
Apolipoprotein B <23 Abetalipoproteinemia/Hypobetalipoproteinemia, 23-45 Hypobetalipoproteinemia  
46-135 Normal >135 [Hyperapobetalipoproteinemia/Increased CAD risk.]  
Extremely low values of ApoB (<48 mg/dL) are related to malabsorption of food lipids and can lead to polyneuropathy.



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### CALCIUM, SERUM

| <u>Investigation</u>  | <u>Result</u> | <u>Units</u> | <u>Biological Reference Interval</u> |
|-----------------------|---------------|--------------|--------------------------------------|
| <b>CALCIUM SERUM</b>  | 9.0           | mg/dL        | 8.8-10.6                             |
| Sample Type :Serum    |               |              |                                      |
| Method : Arsenazo III |               |              |                                      |

### **Interpretation -**

**Increased levels :** Primary and tertiary hyperparathyroidism, malignant diseases, pheochromocytoma (associated parathyroid hyperplasia), sarcoidosis, vitamin D intoxication, milk-alkali syndrome, Paget's disease with immobilization, thyrotoxicosis, acromegaly, diuretic phase of acute tubular necrosis, "idiopathic" hypercalcemia of infancy, dehydration (e.g., from vomiting, diarrhea, alcohol), familial hypocalciuric hypercalcemia, iatrogenic hypercalcemia

**Decreased levels :** Idiopathic, surgical, or congenital hypoparathyroidism; pseudohypoparathyroidism; vitamin D deficiency ; proximal and distal renal tubular disease; alcoholism; hepatic cirrhosis; hypoalbuminemia; neonatal prematurity; inadequate nutrition.



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### C-REACTIVE PROTEIN(CRP), SERUM

| <u>Investigation</u>            | <u>Result</u> | <u>Units</u> | <u>Biological Reference Interval</u> |
|---------------------------------|---------------|--------------|--------------------------------------|
| <b>C-REACTIVE PROTEIN SERUM</b> | < 5.0         | mg/L         | < 5                                  |
| Sample Type :Serum              |               |              |                                      |
| Method : Immunoturbidimetry     |               |              |                                      |

### **Interpretation -**

Acute phase reacting protein. Rapid, marked increases occur with inflammation, infection, trauma and tissue necrosis, malignancies, and autoimmune diseases.  
Concentrations above 100 mg/L are associated with severe stimuli such as major trauma and severe infection (sepsis).



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| <b>Nationality</b>         | : -                     | <b>Sample UID No.</b>     | : D002B042001473   |
| <b>Customer Type</b>       | : Collection Center     | <b>Customer Name</b>      | : AIMS             |
| <b>Ref. Doctor Name</b>    | : -                     | <b>Patient UID No.</b>    | : -- (Other)       |
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### FASTING BLOOD SUGAR(FBS)

| <u>Investigation</u> | <u>Result</u> | <u>Units</u> | <u>Biological Reference Interval</u>                                    |
|----------------------|---------------|--------------|-------------------------------------------------------------------------|
| FASTING BLOOD SUGAR  | 72            | mg/dL        | Normal      =< 100<br>Prediabetes    100 - 125<br>Diabetes        >=126 |

Sample Type :Plasma  
Method : GOD-POD

### **Interpretation -**

Fasting plasma glucose of 100-125 mg/dL is defined as impaired fasting glucose (IFG)  
Fasting plasma glucose of >126 mg/dL after a fast of at least 8 hr on two occasions is diagnostic of diabetes mellitus.



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| <b>Age/DOB/Gender</b>      | : 47Y/1977-01-24/Female | <b>Reported On</b>        | : 30-04-2024 22:43 |
| <b>Nationality</b>         | : -                     | <b>Sample UID No.</b>     | : D002B042001472   |
| <b>Customer Type</b>       | : Collection Center     | <b>Customer Name</b>      | : AIMS             |
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### FERRITIN, SERUM

| <u>Investigation</u>  | <u>Result</u> | <u>Units</u> | <u>Biological Reference Interval</u> |
|-----------------------|---------------|--------------|--------------------------------------|
| <b>FERRITIN SERUM</b> | 50.6          | ng/mL        | 12-150                               |
| Sample Type :Serum    |               |              |                                      |
| Method : CLIA         |               |              |                                      |

### **Interpretation -**

Serum ferritin concentration is a very sensitive and early indicator of iron deficiency that is uncomplicated by other concurrent disease, idiopathic hemochromatosis, and transfusion siderosis. Serum ferritin levels decrease with iron deficiency associated with generalized malnutrition but remain normal in the presence of inflammation associated with iron deficiency. The combination of ferritin and transferrin levels is therefore useful in differentiating these disorders.



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| <b>Patient Id</b>          | : 15206                 | <b>Sample Received On</b> | : 30-04-2024:17:00 |
| <b>Age/DOB/Gender</b>      | : 47Y/1977-01-24/Female | <b>Reported On</b>        | : 30-04-2024 20:44 |
| <b>Nationality</b>         | : -                     | <b>Sample UID No.</b>     | : D002B042001472   |
| <b>Customer Type</b>       | : Collection Center     | <b>Customer Name</b>      | : AIMS             |
| <b>Ref. Doctor Name</b>    | : -                     | <b>Patient UID No.</b>    | : -- (Other)       |
| <b>External Patient Id</b> | : 71367                 |                           |                    |
| <b>Print Version</b>       | : v.1                   |                           |                    |

### IRON, SERUM

| <u>Investigation</u>     | <u>Result</u> | <u>Units</u> | <u>Biological Reference Interval</u> |
|--------------------------|---------------|--------------|--------------------------------------|
| <b>IRON SERUM</b>        | 78            | µg/dL        | 50-170                               |
| Sample Type :Serum       |               |              |                                      |
| Method : Pyridyl azo dye |               |              |                                      |

### **Comments:**

**Increased** : Pernicious, aplastic, and hemolytic anemias; hemochromatosis, acute leukemia, lead poisoning, acute hepatitis, vitamin B6 deficiency, thalassemia, excessive Fe therapy, repeated transfusions, acute Fe poisoning (children), and nephritis  
**Decreased**: Iron-deficiency anemia, remission of PA; acute and chronic infection, carcinoma, nephrosis, hypothyroidism, postoperative state, and kwashiorkor



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|                            |                         |                           |                    |
|----------------------------|-------------------------|---------------------------|--------------------|
| <b>Patient Name</b>        | : MS.CHIANA MIKOVA      | <b>Registered On</b>      | : 30-04-2024 19:50 |
| <b>Patient Id</b>          | : 15206                 | <b>Sample Received On</b> | : 30-04-2024:17:00 |
| <b>Age/DOB/Gender</b>      | : 47Y/1977-01-24/Female | <b>Reported On</b>        | : 30-04-2024 20:44 |
| <b>Nationality</b>         | : -                     | <b>Sample UID No.</b>     | : D002B042001472   |
| <b>Customer Type</b>       | : Collection Center     | <b>Customer Name</b>      | : AIMS             |
| <b>Ref. Doctor Name</b>    | : -                     | <b>Patient UID No.</b>    | : -- (Other)       |
| <b>External Patient Id</b> | : 71367                 |                           |                    |
| <b>Print Version</b>       | : v.1                   |                           |                    |

### Lipase, Serum

| <u>Investigation</u>                    | <u>Result</u> | <u>Units</u> | <u>Biological Reference Interval</u> |
|-----------------------------------------|---------------|--------------|--------------------------------------|
| <b>Lipase Serum</b>                     | 182           | U/L          | 23-300                               |
| Sample Type :Serum                      |               |              |                                      |
| Method : Enzymatic with colipase-VITROS |               |              |                                      |

### **Interpretation -**

In pancreatitis, lipase becomes elevated at about the same time as amylase (4-8 hours). But lipase may rise to a greater extent and remain elevated much longer (7-10 days) than amylase. In acute pancreatitis, normoamylasemia may occur in up to 20% of patients, hence both enzymes should be assayed.



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|                              |                       |                             |                  |
|------------------------------|-----------------------|-----------------------------|------------------|
| <b>Patient Name</b> :        | MS.CHIANA MIKOVA      | <b>Registered On</b> :      | 30-04-2024 19:50 |
| <b>Patient Id</b> :          | 15206                 | <b>Sample Received On</b> : | 30-04-2024:17:00 |
| <b>Age/DOB/Gender</b> :      | 47Y/1977-01-24/Female | <b>Reported On</b> :        | 30-04-2024 20:44 |
| <b>Nationality</b> :         | -                     | <b>Sample UID No.</b> :     | D002B042001472   |
| <b>Customer Type</b> :       | Collection Center     | <b>Customer Name</b> :      | AIMS             |
| <b>Ref. Doctor Name</b> :    | -                     | <b>Patient UID No.</b> :    | -- (Other)       |
| <b>External Patient Id</b> : | 71367                 |                             |                  |
| <b>Print Version</b> :       | v.1                   |                             |                  |

### LIPID PROFILE

| Investigation                                                   | Result        | Units | Biological Reference Interval                                                                          |
|-----------------------------------------------------------------|---------------|-------|--------------------------------------------------------------------------------------------------------|
| <b>TOTAL CHOLESTEROL</b>                                        | 188           | mg/dL | <200 Desirable<br>200-239 Borderline high<br>>240 High                                                 |
| Sample Type :Serum<br>Method : Vitros Microslide                |               |       |                                                                                                        |
| <b>TRIGLYCERIDE SERUM</b>                                       | 134           | mg/dL | Normal: <150<br>Borderline high: 150-199<br>High: 200-499<br>Very high: >500                           |
| Sample Type :Serum                                              |               |       |                                                                                                        |
| <b>HDL CHOLESTEROL</b>                                          | 59            | mg/dL | High risk: < 40<br>Low risk: > 60                                                                      |
| Sample Type :Serum<br>Method : Direct measure, PTA/MgCl2-VITROS |               |       |                                                                                                        |
| <b>LDL CHOLESTEROL</b>                                          | <b>102.20</b> | mg/dL | Optimal: <100<br>Near optimal: 100-129<br>Borderline high: 130-159<br>High: 160-189<br>Very high: >190 |
| Sample Type :Serum<br>Method : Calculated                       |               |       |                                                                                                        |
| <b>VLDL CHOLESTEROL</b>                                         | 26.80         | mg/dL | < 30                                                                                                   |
| Sample Type :Serum<br>Method : Calculated                       |               |       |                                                                                                        |
| <b>NON HDL CHOLESTEROL</b>                                      | 129.00        | mg/dL | Desirable < 130<br>Borderline 130 - 159<br>High >160                                                   |
| Sample Type :Serum<br>Method : Calculated                       |               |       |                                                                                                        |
| <b>TG/HDL Ratio</b>                                             | <b>2.27</b>   | --    | Ideal: <=2.0<br>Good: <=6.0<br>Bad: >6.0                                                               |
| Sample Type :Serum                                              |               |       |                                                                                                        |
| <b>TOTAL CHOLESTEROL HDL RATIO</b>                              | <b>3.19</b>   | --    | Low risk 3.3 - 4.4<br>Average Risk 4.5 - 7.0<br>Moderate Risk 7.1 - 11.0<br>High Risk >11.0            |



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**Patient Name** : MS.CHIANA MIKOVA  
**Patient Id** : 15206  
**Age/DOB/Gender** : 47Y/1977-01-24/Female  
**Nationality** : -  
**Customer Type** : Collection Center  
**Ref. Doctor Name** : -  
**External Patient Id** : 71367  
**Print Version** : v.1

**Registered On** : 30-04-2024 19:50  
**Sample Received On** : 30-04-2024:17:00  
**Reported On** : 30-04-2024 20:44  
**Sample UID No.** : D002B042001472  
**Customer Name** : AIMS  
**Patient UID No.** : -- (Other)

Sample Type :Serum  
Method : Calculated

|                      |      |    |                                                          |
|----------------------|------|----|----------------------------------------------------------|
| <b>LDL HDL ratio</b> | 1.73 | -- | Low Risk < 3.0<br>Borderline 3.1 - 6.0<br>High Risk >6.0 |
|----------------------|------|----|----------------------------------------------------------|

Sample Type :Serum  
Method : Calculated

### Interpretation -

A complete cholesterol test includes the calculation of four types of fats in your blood:

Total cholesterol. This is a sum of your blood's cholesterol content.

Low-density lipoprotein (LDL) cholesterol. This is called the "bad" cholesterol. Too much of it in your blood causes the buildup of fatty deposits (plaques) in your arteries (atherosclerosis), which reduces blood flow. These plaques sometimes rupture and can lead to a heart attack or stroke. High-density lipoprotein (HDL) cholesterol. This is called the "good" cholesterol because it helps carry away LDL cholesterol, thus keeping arteries open and your blood flowing more freely.

Triglycerides. Triglycerides are a type of fat in the blood. When you eat, your body converts calories it doesn't need into triglycerides, which are stored in fat cells. High triglyceride levels are associated with several factors, including being overweight, eating too many sweets or drinking too much alcohol, smoking, being sedentary, or having diabetes with elevated blood sugar levels.



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|                     |                         |                    |                    |
|---------------------|-------------------------|--------------------|--------------------|
| Patient Name        | : MS.CHIANA MIKOVA      | Registered On      | : 30-04-2024 19:50 |
| Patient Id          | : 15206                 | Sample Received On | : 30-04-2024:17:00 |
| Age/DOB/Gender      | : 47Y/1977-01-24/Female | Reported On        | : 30-04-2024 20:44 |
| Nationality         | : -                     | Sample UID No.     | : D002B042001472   |
| Customer Type       | : Collection Center     | Customer Name      | : AIMS             |
| Ref. Doctor Name    | : -                     | Patient UID No.    | : -- (Other)       |
| External Patient Id | : 71367                 |                    |                    |
| Print Version       | : v.1                   |                    |                    |

#### MAGNESIUM, SERUM

| <u>Investigation</u>   | <u>Result</u> | <u>Units</u> | <u>Biological Reference Interval</u> |
|------------------------|---------------|--------------|--------------------------------------|
| <b>MAGNESIUM SERUM</b> | 2             | -            | 1.7-2.3                              |
| Sample Type :Serum     |               |              |                                      |
| Method : Formazon dye  |               |              |                                      |

#### **Interpretation -**

Symptoms of magnesium deficiency do not typically appear until levels are 1.0 mg/dL or lower.  
Levels of 9.0 mg/dL or higher may be life-threatening.

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|                            |                         |                           |                    |
|----------------------------|-------------------------|---------------------------|--------------------|
| <b>Patient Name</b>        | : MS.CHIANA MIKOVA      | <b>Registered On</b>      | : 30-04-2024 19:50 |
| <b>Patient Id</b>          | : 15206                 | <b>Sample Received On</b> | : 30-04-2024:17:00 |
| <b>Age/DOB/Gender</b>      | : 47Y/1977-01-24/Female | <b>Reported On</b>        | : 30-04-2024 20:47 |
| <b>Nationality</b>         | : -                     | <b>Sample UID No.</b>     | : D002B042001475   |
| <b>Customer Type</b>       | : Collection Center     | <b>Customer Name</b>      | : AIMS             |
| <b>Ref. Doctor Name</b>    | : -                     | <b>Patient UID No.</b>    | : -- (Other)       |
| <b>External Patient Id</b> | : 71367                 |                           |                    |
| <b>Print Version</b>       | : v.1                   |                           |                    |

### ERYTHROCYTE SEDIMENTATION RATE

| <u>Investigation</u>                  | <u>Result</u> | <u>Units</u> | <u>Biological Reference Interval</u> |
|---------------------------------------|---------------|--------------|--------------------------------------|
| <b>ERYTHROCYTE SEDIMENTATION RATE</b> | 20            | mm/hr        | 0-20                                 |
| Sample Type :EDTA Whole Blood         |               |              |                                      |
| Method : Modified Westergren Method   |               |              |                                      |

### **Interpretation -**

Moderate elevations are common in active inflammatory disease such as rheumatoid arthritis, chronic infection, collagen disease, and neoplastic disease. ESR has little diagnostic value in these disorders but can be useful in monitoring disease activity. ESR tends to be markedly elevated in monoclonal blood protein disorders such as multiple myeloma or macroglobulinemia, in severe polyclonal hyperglobulinemia due to inflammatory disease, and in hyperfibrinogenemia. ESR is useful and indicated in establishing the diagnosis and in monitoring polymyalgia rheumatica and temporal arteritis, for which the rate typically exceeds 90 mm/hou



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|                            |                         |                           |                    |
|----------------------------|-------------------------|---------------------------|--------------------|
| <b>Patient Name</b>        | : MS.CHIANA MIKOVA      | <b>Registered On</b>      | : 30-04-2024 19:50 |
| <b>Patient Id</b>          | : 15206                 | <b>Sample Received On</b> | : 30-04-2024:17:00 |
| <b>Age/DOB/Gender</b>      | : 47Y/1977-01-24/Female | <b>Reported On</b>        | : 30-04-2024 22:43 |
| <b>Nationality</b>         | : -                     | <b>Sample UID No.</b>     | : D002B042001472   |
| <b>Customer Type</b>       | : Collection Center     | <b>Customer Name</b>      | : AIMS             |
| <b>Ref. Doctor Name</b>    | : -                     | <b>Patient UID No.</b>    | : -- (Other)       |
| <b>External Patient Id</b> | : 71367                 |                           |                    |
| <b>Print Version</b>       | : v.1                   |                           |                    |

### CA- 125

| Investigation      | Result | Units | Biological Reference Interval |
|--------------------|--------|-------|-------------------------------|
| <b>CA- 125</b>     | 7.97   | U/ml  | 0-35                          |
| Sample Type :Serum |        |       |                               |
| Method : CLIA      |        |       |                               |

### Interpretation -

An aid in the management of Ovarian cancer patients. Preoperative CA 125 level of < 65 U /mL is associated with a significantly greater 5 year survival rate.

Monitor the course of disease in patients with Invasive epithelial ovarian cancer

Detection of residual tumor in patients with Primary epithelial ovarian cancer who have undergone first line therapy. Persistent elevation of CA 125 levels after 3 cycles of therapy indicates a poor prognosis. CA125 – CA 125 is elevated in approximately 50% patients with early stage disease and >80% patients with advanced disease. Thus serial CA 125 levels over time may be beneficial as a screening tool.

Limitation:

Normal concentration of CA125 does not exclude presence of the tumor.

Even at high concentrations, CA125 alone is not useful for distinguishing benign from malignant pelvic masses.

Note: Human epididymis protein 4 (HE4) – helpful in diagnosing recurrent or progressive disease or in the evaluation of a suspicious adnexal mass. This is an FDA approved test for monitoring the disease

### Comments:

For the entire male population as per NIH guideline, the mean CA125 value is 7.50 U/mL OR kU/L and 2.5th-97.5th percentiles reference range is 2.40-13.2 U/mL OR kU/L, respectively.



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|                            |                         |                           |                    |
|----------------------------|-------------------------|---------------------------|--------------------|
| <b>Patient Name</b>        | : MS.CHIANA MIKOVA      | <b>Registered On</b>      | : 30-04-2024 19:50 |
| <b>Patient Id</b>          | : 15206                 | <b>Sample Received On</b> | : 30-04-2024:17:00 |
| <b>Age/DOB/Gender</b>      | : 47Y/1977-01-24/Female | <b>Reported On</b>        | : 01-05-2024 00:57 |
| <b>Nationality</b>         | : -                     | <b>Sample UID No.</b>     | : D002B042001472   |
| <b>Customer Type</b>       | : Collection Center     | <b>Customer Name</b>      | : AIMS             |
| <b>Ref. Doctor Name</b>    | : -                     | <b>Patient UID No.</b>    | : -- (Other)       |
| <b>External Patient Id</b> | : 71367                 |                           |                    |
| <b>Print Version</b>       | : v.1                   |                           |                    |

### PROLACTIN

| <u>Investigation</u> | <u>Result</u> | <u>Units</u> | <u>Biological Reference Interval</u>                                     |
|----------------------|---------------|--------------|--------------------------------------------------------------------------|
| <b>PROLACTIN</b>     | 4.32          | ng/mL        | Male: 3.0-14.7<br>Female: 3.8-23.2<br>Pregnancy, third trimester: 95-473 |

Sample Type :Serum  
Method : CLIA

### **Interpretation -**

**Increased** : Prolactin-secreting pituitary tumors (macroadenomas, PRL frequently >1000 ng/mL; in microadenoma, PRL >150 ng/mL; three quarters of patients with amenorrhea and/or galactorrhea, Chiari-Frommel and Argonz Del Castillo syndromes; various types of hypothalamic-pituitary diseases (e.g., sarcoidosis, granulomatous diseases, craniopharyngioma, metastatic cancer, empty sella syndrome), primary hypothyroidism, anorexia nervosa, polycystic ovary syndrome, renal failure, insulin-induced hypoglycemia, chest wall injury, adrenal insufficiency, and surgery (e.g., pituitary stalk section)

**Decreased** : Pituitary apoplexy (Sheehan's syndrome)



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**Patient Name** : MS.CHIANA MIKOVA  
**Patient Id** : 15206  
**Age/DOB/Gender** : 47Y/1977-01-24/Female  
**Nationality** : -  
**Customer Type** : Collection Center  
**Ref. Doctor Name** : -  
**External Patient Id** : 71367  
**Print Version** : v.1

**Registered On** : 30-04-2024 19:50  
**Sample Received On** : 30-04-2024:17:00  
**Reported On** : 30-04-2024 21:31  
**Sample UID No.** : D002B042001475  
**Customer Name** : AIMS  
**Patient UID No.** : -- (Other)

### HAEMOGLOBIN A1C(HbA1C)

| <u>Investigation</u>   | <u>Result</u> | <u>Units</u> | <u>Biological Reference Interval</u>                                                                                                      |
|------------------------|---------------|--------------|-------------------------------------------------------------------------------------------------------------------------------------------|
| HAEMOGLOBIN A1C(HbA1C) | 5.5           | %            | Non-Diabetic: < 5.7<br>Pre-Diabetic: 5.7 - 6.4<br>Diabetic:-<br>Good Control: 6.0 - 7.0<br>Fair control: 7.1 - 8.0<br>Poor Control: > 8.0 |

Sample Type :EDTA Whole Blood  
Method : HPLC

### **Interpretation -**

Achieving A1C targets of <7% (has been shown to reduce microvascular complications of type 1 and type 2 diabetes when instituted early in the course of disease, the greatest number of complications will be averted by taking patients from very poor control to fair/good control.



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**Patient Name** : MS.CHIANA MIKOVA  
**Patient Id** : 15206  
**Age/DOB/Gender** : 47Y/1977-01-24/Female  
**Nationality** : -  
**Customer Type** : Collection Center  
**Ref. Doctor Name** : -  
**External Patient Id** : 71367  
**Print Version** : v.1

**Registered On** : 30-04-2024 19:50  
**Sample Received On** : 30-04-2024:17:00  
**Reported On** : 30-04-2024 20:43  
**Sample UID No.** : D002B042001474  
**Customer Name** : AIMS  
**Patient UID No.** : -- (Other)

### URINE ANALYSIS

| Investigation                           | Result                               | Units | Biological Reference Interval |
|-----------------------------------------|--------------------------------------|-------|-------------------------------|
| <b><u>PHYSICAL EXAMINATION</u></b>      |                                      |       |                               |
| COLOUR                                  | Pale Yellow                          | --    | Pale Yellow                   |
| APPEARANCE                              | CLEAR                                | --    | CLEAR                         |
| VOLUME                                  | 50                                   | ml    | -                             |
| <b><u>CHEMICAL EXAMINATION :</u></b>    |                                      |       |                               |
| SPECIFIC GRAVITY                        | 1.010                                | --    | 1.005 - 1.030                 |
| REACTION (pH)                           | 8.0                                  | --    | --                            |
| GLUCOSE                                 | ABSENT                               | --    | ABSENT                        |
| PROTEIN                                 | ABSENT                               | --    | ABSENT                        |
| BILIRUBIN                               | ABSENT                               | --    | ABSENT                        |
| UROBILINOGEN                            | NORMAL                               | --    | NORMAL                        |
| KETONES                                 | ABSENT                               | --    | ABSENT                        |
| NITRITE                                 | NEGATIVE                             | --    | NEGATIVE                      |
| LEUCOCYTES ESTERASE                     | NEGATIVE                             | --    | NEGATIVE                      |
| BLOOD                                   | ABSENT                               | --    | ABSENT                        |
| <b><u>MICROSCOPIC EXAMINATION :</u></b> |                                      |       |                               |
| PUS CELLS                               | 0 - 2                                | /hpf  | 0 - 5                         |
| EPITHELIAL CELLS                        | 2 - 4                                | /hpf  | 0 - 5                         |
| RBCs                                    | ABSENT                               | /hpf  | ABSENT                        |
| CAST                                    | Nil                                  | /hpf  | Nil                           |
| CRYSTALS                                | AMORPHOUS<br>PHOSPHATES -<br>PRESENT | --    | Nil                           |
| OTHERS                                  | BACTERIA -<br>PRESENT                | /hpf  | NOT SEEN                      |

**Interpretation -**



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|                     |                         |                    |                    |
|---------------------|-------------------------|--------------------|--------------------|
| Patient Name        | : MS.CHIANA MIKOVA      | Registered On      | : 30-04-2024 19:50 |
| Patient Id          | : 15206                 | Sample Received On | : 30-04-2024:17:00 |
| Age/DOB/Gender      | : 47Y/1977-01-24/Female | Reported On        | : 30-04-2024 20:43 |
| Nationality         | : -                     | Sample UID No.     | : D002B042001474   |
| Customer Type       | : Collection Center     | Customer Name      | : AIMS             |
| Ref. Doctor Name    | : -                     | Patient UID No.    | : -- (Other)       |
| External Patient Id | : 71367                 |                    |                    |
| Print Version       | : v.1                   |                    |                    |

Methodology: DIP STICK & MICROSCOPY

#### Comments:

Urine analysis helps diagnose medical conditions, including urinary tract infections, kidney diseases, diabetes, liver problems, and certain metabolic disorders. Used as screening tool indicating signs of dehydration, drug use, presence of blood or abnormal levels of specific substances in the body. Monitoring treatment and in pregnancy. Also useful in early disease detection and overall health assessment etc.

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| <b>Patient Name</b>        | : MS.CHIANA MIKOVA      | <b>Registered On</b>      | : 30-04-2024 19:50 |
| <b>Patient Id</b>          | : 15206                 | <b>Sample Received On</b> | : 30-04-2024:17:00 |
| <b>Age/DOB/Gender</b>      | : 47Y/1977-01-24/Female | <b>Reported On</b>        | : 30-04-2024 20:44 |
| <b>Nationality</b>         | : -                     | <b>Sample UID No.</b>     | : D002B042001472   |
| <b>Customer Type</b>       | : Collection Center     | <b>Customer Name</b>      | : AIMS             |
| <b>Ref. Doctor Name</b>    | : -                     | <b>Patient UID No.</b>    | : -- (Other)       |
| <b>External Patient Id</b> | : 71367                 |                           |                    |
| <b>Print Version</b>       | : v.1                   |                           |                    |

### RENAL FUNCTION TEST

| Investigation                                                                                      | Result       | Units         | Biological Reference Interval |
|----------------------------------------------------------------------------------------------------|--------------|---------------|-------------------------------|
| <b>BLOOD UREA</b><br>Sample Type :Serum<br>Method : Urease, colorimetric                           | 42           | mg/dL         | 12.84-42.8                    |
| <b>BLOOD UREA NITROGEN(BUN)</b><br>Sample Type :Serum<br>Method : Urease, colorimetric             | 19.63        | mg/dL         | 6-20                          |
| <b>CREATININE SERUM</b><br>Sample Type :Serum<br>Method : Enzymatic-VITROS, IFCC-IDMS Standardized | 0.7          | mg/dL         | 0.6-1.12                      |
| <b>URIC ACID SERUM</b><br>Sample Type :Serum<br>Method : URICASE, ENZYMATIC COLORIMETRIC           | 4.0          | mg/dL         | 2.5-6.2                       |
| <b>e-GFR</b><br>Sample Type :Serum                                                                 | 90           | mL/min/1.73m2 | 75-190                        |
| <b>BUN CREATININE RATIO</b><br>Sample Type :Serum<br>Method : Calculated                           | <b>28.04</b> | --            | 10-20                         |

### Interpretation -

Interpretation of renal function tests requires considering multiple factors, including the patient's age, sex, muscle mass, medications, and clinical history. It's important to note that renal function tests are not diagnostic on their own and are often used in conjunction with other clinical assessments and imaging studies to evaluate kidney function comprehensively.

Abnormal results may indicate various kidney conditions, including acute or chronic kidney disease, glomerulonephritis, kidney infections, kidney stones, and renal tubular disorders. They can also point to non-renal conditions such as heart failure, liver disease, or dehydration.



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|                            |                         |                           |                    |
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| <b>Nationality</b>         | : -                     | <b>Sample UID No.</b>     | : D002B042001472   |
| <b>Customer Type</b>       | : Collection Center     | <b>Customer Name</b>      | : AIMS             |
| <b>Ref. Doctor Name</b>    | : -                     | <b>Patient UID No.</b>    | : -- (Other)       |
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### VITAMIN B12

| <u>Investigation</u> | <u>Result</u> | <u>Units</u> | <u>Biological Reference Interval</u> |
|----------------------|---------------|--------------|--------------------------------------|
| <b>VITAMIN B12</b>   | 201           | pg/mL        | 200-1100                             |
| Sample Type :Serum   |               |              |                                      |
| Method : ECLIA       |               |              |                                      |

### **Interpretation -**

Vitamin B12 deficiency frequently causes macrocytic anemia, glossitis, peripheral neuropathy, weakness, hyperreflexia, ataxia, loss of proprioception, poor coordination, and affective behavioral changes. These manifestations may occur in any combination; many patients have the neurologic defects without macrocytic anemia.

Vitamin B12 deficiency may be due to lack of IF secretion by gastric mucosa (eg, gastrectomy, gastric atrophy) or intestinal malabsorption (eg, ileal resection, small intestinal diseases). Pernicious anemia is a macrocytic anemia caused by vitamin B12 deficiency that is due to a lack of IF secretion by gastric mucosa.

### **Comments:**

Serum methylmalonic acid and homocysteine levels are also elevated in vitamin B12 deficiency states



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### VITAMIN D3

| <u>Investigation</u> | <u>Result</u> | <u>Units</u> | <u>Biological Reference Interval</u>                                                  |
|----------------------|---------------|--------------|---------------------------------------------------------------------------------------|
| VITAMIN D3           | 36.6          | ng/mL        | Deficiency: <10<br>Insufficiency: 10 - 29<br>Sufficiency: 30 - 100<br>Toxicity: > 100 |

Sample Type :Serum  
Method : CLIA

### **Interpretation -**

Studies show that 25-hydroxyvitamin D2 and D3 (25-OH-VitD) levels below 25 ng/mL are associated with an increased risk of secondary hyperparathyroidism, reduced bone mineral density, and fractures, particularly in the elderly. Intervention studies support this clinical cutoff, showing a reduction of fracture risk with 25-OH-VitD replacement.

Levels less than 10 ng/mL may be associated with more severe abnormalities and can lead to inadequate mineralization of newly formed osteoid, resulting in rickets in children and osteomalacia in adults. In these individuals, serum calcium levels may be marginally low, and parathyroid hormone (PTH) and serum alkaline phosphatase are usually elevated. Definitive diagnosis rests on the typical radiographic findings or bone biopsy/histomorphometry.

### **Comments:**

Baseline biochemical work-up of suspected cases of rickets and osteomalacia should include measurement of serum calcium, phosphorus, PTH, and 25-OH-VitD. In patients where testing is not completely consistent with the suspected diagnosis, in particular, if serum 25-OH-VitD levels are greater than 10 ng/mL, an alternative cause for impaired mineralization should be considered. Possible differential diagnosis includes: partly treated vitamin D deficiency, extremely poor calcium intake, vitamin D resistant rickets, renal failure, renal tubular mineral loss with or without renal tubular acidosis, hypophosphatemic disorders (eg, X-linked or autosomal dominant hypophosphatemic rickets), congenital hypoparathyroidism, activating calcium sensing receptor mutations, and osteopetrosis. Measurement of serum urea, creatinine, magnesium, and 1,25-dihydroxyvitamin D (DHVD) is recommended as a minimal additional workup for these patients.



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#### THYROID FUNCTION TEST (TOTAL)

| Investigation                                          | Result | Units  | Biological Reference Interval |
|--------------------------------------------------------|--------|--------|-------------------------------|
| <b>TOTAL T3</b><br>Sample Type :Serum<br>Method : CLIA | 1.1    | ng/mL  | 0.8-2.15                      |
| <b>TOTAL T4</b><br>Sample Type :Serum<br>Method : CLIA | 58.6   | ug/dl  | 52-127                        |
| <b>TSH</b><br>Sample Type :Serum<br>Method : CLIA      | 0.82   | μIU/ml | 0.3-4.5                       |

#### Interpretation -

Serum T3 (triiodothyronine) levels often are depressed in sick and hospitalized patients, caused in part by the biochemical shift to the production of reverse T3. Therefore, T3 generally is not a reliable predictor of hypothyroidism. However, in a small subset of hyperthyroid patients, hyperthyroidism may be caused by overproduction of T3 (T3 toxicosis). To help diagnose and monitor this subgroup, T3 is measured on all specimens with suppressed TSH and normal free T4 concentrations.

Detectable concentrations of antithyroperoxidase (anti-TPO) antibodies are observed in patients with autoimmune thyroiditis and may cause the destruction of thyroid tissue, eventually resulting in hypothyroidism. Anti-TPO antibodies are measured in all specimens with elevated TSH concentrations.



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| <b>Customer Type</b>       | : Collection Center     | <b>Customer Name</b>      | : AIMS             |
| <b>Ref. Doctor Name</b>    | : -                     | <b>Patient UID No.</b>    | : -- (Other)       |
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#### ELECTROLYTES (Na/K/Cl)

| <u>Investigation</u>                                                | <u>Result</u> | <u>Units</u> | <u>Biological Reference Interval</u> |
|---------------------------------------------------------------------|---------------|--------------|--------------------------------------|
| <b>SODIUM SERUM</b><br>Sample Type :Serum<br>Method : ISE direct    | 140           | mEq/L        | 135-145                              |
| <b>POTASSIUM SERUM</b><br>Sample Type :Serum<br>Method : ISE direct | 4.9           | mEq/L        | 3.6-5.2                              |
| <b>CHLORIDE SERUM</b><br>Sample Type :Serum<br>Method : ISE direct  | 104           | mEq/L        | 98-107                               |

#### **Interpretation -**

With an imbalance of a single electrolyte, such as sodium or potassium, repeat testing may be ordered of that particular electrolyte, can be used to monitor the imbalance until remedied. With an acid-base imbalance, blood gases may be ordered, which will measure the oxygen, carbon dioxide, and pH levels in the arterial blood.

#### **Comments:**

Electrolyte concentrations are evaluated to assist in investigating conditions that cause electrolyte imbalances such as dehydration, kidney disease, lung diseases, or heart conditions.



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#### % TRANSFERRIN SATURATION

| <u>Investigation</u>          | <u>Result</u> | <u>Units</u> | <u>Biological Reference Interval</u> |
|-------------------------------|---------------|--------------|--------------------------------------|
| <b>TRANSFERRIN SATURATION</b> | 25.16         | %            | 15-50                                |
| Sample Type :Serum            |               |              |                                      |

#### **Interpretation -**

Transferrin is the primary plasma iron transport protein, which binds iron strongly at physiological pH. Transferrin is generally only 25% to 30% saturated with iron. The additional amount of iron that can be bound is the unsaturated iron-binding capacity (UIBC).

Percent saturation (100 x serum iron/TIBC) is usually normal or decreased in persons who are iron deficient, pregnant, or are taking oral contraceptive medications. Persons with chronic inflammatory processes, hemochromatosis, or malignancies generally display low transferrin.

Serum iron, total iron-binding capacity, and percent saturation are widely used for the diagnosis of iron deficiency. However, serum ferritin is a much more sensitive and reliable test for demonstration of iron deficiency.



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#### TOTAL IRON BINDING CAPACITY

| <u>Investigation</u>               | <u>Result</u> | <u>Units</u> | <u>Biological Reference Interval</u> |
|------------------------------------|---------------|--------------|--------------------------------------|
| <b>TOTAL IRON BINDING CAPACITY</b> | 310           | µg/dL        | 240-450                              |
| Sample Type :Serum                 |               |              |                                      |

#### **Comments:**

Total Iron Binding Capacity is an essential laboratory parameter used to assess iron metabolism, diagnose iron-related disorders, differentiate types of anemia, evaluate iron absorption disorders, and monitor the effectiveness of iron therapy.



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|                              |                       |                             |                  |
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| <b>Customer Type</b> :       | Collection Center     | <b>Customer Name</b> :      | AIMS             |
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### Homocysteine

| Investigation                | Result      | Units  | Biological Reference Interval |
|------------------------------|-------------|--------|-------------------------------|
| <b>Homocysteine</b>          | <b>45.5</b> | umol/L | 4.390-14.650                  |
| Sample Type :Serum           |             |        |                               |
| Method : CLIA                |             |        |                               |
| Test Comment : Value checked |             |        |                               |

### Interpretation -

| Fasting/basal tHcy |                     |                 |
|--------------------|---------------------|-----------------|
| Group              | Folate supplemented | Nonsupplemented |
| Pregnancy          | 8                   | 10              |
| < 15 Years         | 8                   | 10              |
| 15-65 Years        | 12                  | 15              |
| > 65 Years         | 16                  | 20              |

Useful as :1. An aid for screening patients suspected of having an inherited disorder of methionine metabolism including:-Cystathionine beta-synthase deficiency (homocystinuria), -Methylenetetrahydrofolate reductase deficiency and its thermolabile variants, -Methionine synthase deficiency, -Cobalamin (Cbl) metabolism,-Combined methyl-Cbl and adenosyl-Cbl deficiencies: Cbl C2, Cbl D2, and Cbl F3 deficiencies, -Methyl-Cbl specific deficiencies: Cbl D-Var1, Cbl E, and Cbl G deficiencies, -Transcobalamin II deficiency, -Adenosylhomocysteinase deficiency, -Glycine N-methyltransferase deficiency, -Methionine adenosyltransferase I/III deficiency

- 2.Screening and monitoring patients suspected of, or confirmed with, an inherited disorder of methionine metabolism
- 3.Evaluating individuals with suspected deficiency of vitamin B12 or folate

### Comments:

Elevated homocysteine concentrations are considered informative in patients evaluated for suspected nutritional deficiencies (vitamin B12, folate) and inborn errors of metabolism. Measurement of methylmalonic acid (MMA) distinguishes between vitamin B12 (cobalamin) and folate deficiencies, as MMA is only elevated in vitamin B12 deficiency. Treatment response can be evaluated by monitoring plasma homocysteine concentrations over time.



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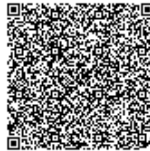
### Apolipoprotein B/A1 Ratio

| <u>Investigation</u>             | <u>Result</u> | <u>Units</u> | <u>Biological Reference Interval</u>                                                                      |
|----------------------------------|---------------|--------------|-----------------------------------------------------------------------------------------------------------|
| <b>Apolipoprotein B/A1 Ratio</b> | 0.57          | Ratio        | 2 - 17 years: < 0.8<br>>= 18 years:<br>Lower risk: < 0.7<br>Average Risk: 0.7 - 0.9<br>Higher risk: > 0.9 |

--End Of Report--

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